

Difficulty Swallowing

(Dysphagia)

By **Jonathan Gotfried**, MD, Main Line Health, Bryn Mawr, PA
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Some people have difficulty swallowing (dysphagia). In dysphagia, foods, liquids, or both do not move normally from the throat (pharynx) to the stomach. People feel as though food or liquids become stuck on the way down the tube that connects the throat to the stomach (esophagus).

Dysphagia should not be confused with lump in throat ([globus sensation](#)), in which people have the sensation of a lump in their throat but have no difficulty swallowing.

Complications of difficulty swallowing

Dysphagia can cause people to inhale (aspirate) mouth secretions and/or material they eat or drink. Aspiration can cause [acute pneumonia](#). If aspiration occurs over a long period of time, people may develop chronic lung disease.

People who have had dysphagia for a long time are often inadequately nourished and lose weight.

Causes of Difficulty Swallowing

Although most people take swallowing for granted, it is actually a complicated process. For swallowing to take place normally, the brain must unconsciously coordinate the activity of numerous small muscles of the throat and the esophagus. These muscles must contract strongly and in the proper sequence to push food from the mouth to the back of the throat and then down the esophagus. The upper part of the esophagus must relax to allow food to enter the esophagus. Finally, the lower part

of the esophagus must relax to allow food to enter the stomach. Thus, swallowing difficulty can result from the following:

- Disorders of the brain or nervous system
- Disorders of the muscles in general
- Disorders of the esophagus (a physical blockage or a motility [movement] disorder)

Brain and nervous system disorders that cause difficulty swallowing include [stroke](#), [Parkinson disease](#), [multiple sclerosis](#), and [amyotrophic lateral sclerosis](#) (ALS). People with these disorders typically have other symptoms in addition to difficulty swallowing. Many have already been diagnosed with these disorders.

General muscle disorders that cause difficulty swallowing include [myasthenia gravis](#), [dermatomyositis](#), and [muscular dystrophy](#).

A physical blockage of the esophagus can result from [cancer of the esophagus](#), [rings](#) or [webs](#) of tissue across the inside of the esophagus, and scarring of the esophagus from chronic acid reflux or from swallowing a caustic liquid. [Eosinophilic esophagitis](#) (inflammation of the esophagus caused by a higher than normal number of eosinophils, a specific type of white blood cell) causes dysphagia due to narrowing (stricture) of the esophagus. Sometimes the esophagus is compressed by a nearby organ or structure such as an enlarged thyroid gland, a bulge in the large artery in the chest (aortic aneurysm), or a tumor in the middle of the chest.

Esophageal motility disorders include [achalasia](#) (in which the rhythmic contractions of the esophagus are greatly decreased and the lower esophageal muscle does not relax normally to allow food to pass into the stomach) and [esophageal spasm](#). [Systemic sclerosis](#) (scleroderma) may also cause a motility disorder.

Evaluation of Difficulty Swallowing

Not every episode of dysphagia requires immediate evaluation by a doctor. The following information can help people decide when a doctor's evaluation is needed and help them know what to expect during the evaluation.

Warning signs

In people with dysphagia, certain symptoms and characteristics are cause for concern. They include:

- Symptoms of a complete physical blockage (such as drooling or inability to swallow anything at all)
- Dysphagia resulting in weight loss
- Painful swallowing (odynophagia)
- A new problem in nerve, spinal cord, or brain function, particularly any weakness
- Recurring [aspiration pneumonia](#) (lung infection caused by inhaling mouth secretions, stomach contents, or both)

When to see a doctor

People who have warning signs should see a doctor right away unless the only warning sign is weight loss. In such cases, a delay of a week or so is not harmful.

People with dysphagia but no warning signs should see their doctor within a week or so. However, people who cough or choke whenever they eat or drink should be evaluated sooner.

What the doctor does

Doctors first ask questions about the person's symptoms and medical history. Doctors then do a physical examination. What they find during the history and physical examination often suggests a cause of the dysphagia and the tests that may need to be done (see table [Some Causes and Features of Swallowing Difficulty](#))

During the history, doctors ask about the following:

- Any difficulty swallowing solids, liquids, or both
- Food coming out the nose
- Drooling or food spilling from the mouth
- Coughing or choking while eating

People with equal difficulty swallowing liquids and solids are more likely to have an esophageal motility disorder. People who have gradually increasing difficulty swallowing first solids and then liquids may have a worsening physical blockage, such as a tumor. Food unintentionally coming out of the nose or mouth suggests a neurologic or muscular problem rather than a problem with the esophagus.

Doctors look for symptoms that suggest neuromuscular, gastrointestinal, and connective tissue disorders. Major neuromuscular symptoms include weakness, either constant weakness of a body part (such as an arm or leg) or off-and-on weakness that occurs during activity and is relieved by rest; walking (gait) or balance disturbance; involuntary, rhythmic, shaking movements (tremors); and difficulty speaking. Doctors also need to know whether the person has a known disease that causes dysphagia (see table [Some Causes and Features of Swallowing Difficulty](#))

Doctors then do a physical examination. The physical examination is focused on the neurologic examination, but doctors also pay attention to the person's nutritional status and any abnormalities of the skin and muscles. During the physical examination, doctors look at the following:

- Tremors present while the person is at rest
- Muscle strength (including muscles of the eyes, mouth, and face)
- The person's voice (hoarse or weak, or unable to speak or sing aloud) by having them say a few words or sing a few notes of their favorite song
- The way the person walks (normal, shuffling, or with a wide-based gait)
- The skin (normal, pale, or discolored; dry or moist; thick or thin; the fingernails normal or brittle)
- Muscles (normal or wasted; tender or not tender)
- The neck (normal or swollen)



Some Causes and Features of Swallowing Difficulty

Cause	Common Features*	Tests
Neurologic disorders		
Stroke	Usually a previous diagnosis of a stroke Weakness or paralysis on one side of the body, difficulty speaking, difficulty walking, or a combination	CT or MRI of the brain
Parkinson disease	Muscle stiffness and fewer voluntary movements than normal Involuntary, rhythmic, shaking movements (tremors), incoordination (ataxia), and balance disturbance	A doctor's examination Sometimes CT or MRI
Multiple sclerosis	Come-and-go symptoms involving various parts of the body, including vision problems, muscle weakness, and/or abnormal sensations Sometimes weak, clumsy movements	MRI Often a spinal tap
Some motor neuron disorders, such as: • Amyotrophic lateral sclerosis	Muscle twitching, wasting, and weakness Progressive difficulty with chewing,	Electrodiagnostic tests (such as needle electromyography, which involves stimulating muscles



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